*** DRAFT REPORT - REQUIRES CLINICIAN REVIEW ***

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PATIENT: Marcus Vance | ID: PAT-1003 | DATE: 2026-10-18 00:48:07

STATUS: DRAFT_PENDING_REVIEW | CLINICIAN SIGN-OFF REQUIRED: YES

RAW CLINICAL DICTATION / TRANSCRIPT:

"Tooth #29 shows apical rarefying and sclerosing osteitis. TMJs show bilateral condyla"
"r flattening consistent with degenerative joint changes. Left palatine tonsillolith n"
"oted incidentally."

STRUCTURED ANATOMICAL FINDINGS:

1. Site: Tooth 45 (Mandibular right second premolar) [Periapical / Osseous Pathology]
Finding: Tooth #29 shows apical rarefying and sclerosing osteitis
Diagnostic Certainty: definite observation

2. Site: TMJ [Temporomandibular Joint]
Finding: TMJs show bilateral condylar flattening consistent with degenerative joint changes
Diagnostic Certainty: suspected / differential diagnosis

3. Site: Tonsillolith [Sinonasal & Soft Tissue]
Finding: Left palatine tonsillolith noted incidentally
Diagnostic Certainty: definite observation

CLINICAL IMPRESSIONS:

1. Tooth 45 (Mandibular right second premolar): Tooth #29 shows apical rarefying and sclerosing osteitis. Findings suggestive of periapi
2. TMJ: Degenerative osseous changes / remodeling.
3. Tonsillolith: Incidental soft tissue finding.

RECOMMENDATIONS & MANDATORY REVIEW:

1. Correlate with dental pulp vitality testing, percussion sensitivity, and clinical symptom history.
2. Clinical correlation with jaw mobility, joint noises (crepitus/clicking), and pain symptoms.
3. No acute surgical intervention required on imaging alone; monitor during routine reviews.
4. MANDATORY CLINICIAN ACTION: Attending dentist/radiologist must review and verify all anatomical labels and findings before clini

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CLINICIAN SIGNATURE & VERIFICATION BLOCK:

Attending Radiologist / Dentist Signature: _____ Date: _____